

IMPORTANT: Patient Report enclosed

The Oncotype DX® test report for the following patient is now available:

Patient Name	CORNELIA IOVAN
Patient ID	RO-PT6D5CE6
Patient Date of Birth	20/11/1975
Ordering Physician	Rodica Anghel
Requisition	RO-DEBBA8

Should you have any questions regarding this report, please do not hesitate to contact Customer Support.

Exact Sciences Customer Support

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Email: europiansupport@exactsciences.com

CONFIDENTIAL PATIENT INFORMATION ENCLOSED

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Oncotype DX Breast Recurrence Score® Report

Node Positive (N1mic and N1)

EXACT
SCIENCES

Premenopausal

RO-PT6D5CE6

Date of Birth: 20-Nov-1975 Gender: Female Report Number: ORE01605065-01 Report Date: 24-Apr-2025
Specimen Source/ID: Breast/224454 9 Date of Collection: 04-Mar-2025
Ordering Physician: Dr. Anghel Rodica

**Recurrence Score®
(RS) Result**

16

Clinical factors may be considered with the RS when making individual treatment decisions.

Distant Recurrence Risk at 5 Years

ET Alone

6%

95% CI (5%, 8%)

RxPONDER¹

ET = Endocrine Therapy
CI = Confidence Intervals

Group Average Absolute Chemotherapy (CT) Benefit*

RS 14-25

2.7%

95% CI (-0.5%, 5.9%)

RxPONDER²

*It is not known whether the benefit observed with chemotherapy is due to chemotherapy alone, ovarian function suppression or a combination of the two.

Quantitative Single-Gene Scores^{3,4}



RxPONDER Study^{1,5}

Results in this report are based on the Recurrence Score (RS) and data from the SWOG S1007 study, known as RxPONDER.^{1,5} RxPONDER is a prospective, multinational phase III clinical trial that enrolled 5,083 women (≥18 years) with hormone receptor-positive (HR+), HER2-negative invasive breast cancer and 1-3 positive axillary lymph nodes. These patients were randomized to receive endocrine therapy alone or chemotherapy followed by endocrine therapy. The primary endpoint of the RxPONDER trial was 5-year Invasive Disease-Free Survival (IDFS).

Premenopausal women with a RS of 0-25 showed a 2.4% absolute benefit of chemotherapy in addition to endocrine therapy for the endpoint of 5-year Distant Recurrence-Free Interval (DRFI) (competing risk adjusted HR = 0.64, 95% CI: 0.43-0.95, p = 0.026). Consistent chemotherapy benefit was also observed for Invasive Disease-Free Survival for subgroups of age, tumor size, grade, Recurrence Score result, and number of positive lymph nodes, with the exception of women ≥50 years who were considered premenopausal.

The individualized **Distant Recurrence Risk** at 5 years is for premenopausal patients with a RS 0-25 who were treated with endocrine therapy alone. The group average **Absolute Benefit of Chemotherapy** for premenopausal women with a RS 14-25 was 2.7%.²

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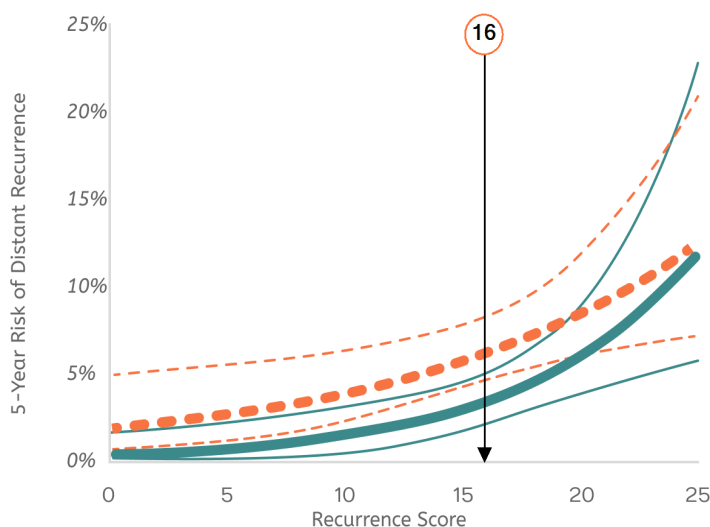
Medical Record/Patient #:

Client: GeneKor Medical (Romania)

Specimen Received: 18-Apr-2025

Additional Recipient: Dr. Georgios Nasioulas

RxPONDER²



ET+CHEMO

95% CI

ET ALONE

95% CI

Methods and Limitations

The Oncotype DX Breast Recurrence Score test uses RT-PCR to provide information on prognosis and the magnitude of chemotherapy benefit to guide chemotherapy treatment decisions in patients with early-stage, HR+, HER2-negative and lymph node-negative or lymph node-positive (N1) breast cancer. Decisions on treatment should also be based on independent medical judgment of the treating physician taking into consideration all available information concerning the patient's medical condition, in accordance with your community's standard of care.

The **Recurrence Score (RS) Result** which ranges from 0-100 is calculated from the quantitative RT-PCR analysis of 21 specific genes.

Quantitative Single-Gene Scores for quality control. The Oncotype DX test uses quantitative RT-PCR to determine the RNA expression of ER, PR and HER2, using the published validated cut-offs.^{3,4} The standard deviations of single-gene results are less than 0.5 units. The RT-PCR single-gene results may differ from ER, PR, or HER2 results reported using other methods or reported by other laboratories.

The definitions of **menopausal status** based upon the RxPONDER⁵ trial are as follows:

- **Premenopausal:** Less than 6 months since last menstrual period and no prior bilateral oophorectomy and not on estrogen replacement.
- **Postmenopausal:** Prior bilateral oophorectomy or more than 12 months since last menstrual period with no prior hysterectomy.
- If these definitions did not apply, patients were categorized as premenopausal if <50 years and postmenopausal if ≥50 years.

Distant Recurrence-Free Interval (DRFI): Time from randomization to distant recurrence or death from breast cancer.

Invasive Disease-Free Survival (IDFS): Time from randomization to invasive breast cancer recurrence (local, regional, or distant), second invasive primary cancer (breast cancer or not) or death from any cause.

References:

1. Kalinsky K, et al. San Antonio Breast Cancer Symposium 2021. 2. Data on File. 3. Badve et al. *J Clin Oncol*. 2008. 4. Baehner et al. *J Clin Oncol*. 2010. 5. Kalinsky et al. *NEJM*. 2021.

Laboratory Director(s): NhuThuy Thi Can, MD

This test was developed and its performance characteristics determined by Genomic Health, Inc. It has not been cleared or approved by the FDA, nor is it currently required to be. The laboratory is regulated under CLIA and qualified to perform high-complexity testing. This test is used for clinical purposes. It should not be regarded as investigational or for research.

The Oncotype DX Breast Recurrence Score Test is an in vitro diagnostic device, CE marked under Regulation (EU) 2017/46 of the European Parliament and of the Council of 5 April 2017 on in vitro diagnostic medical devices. In Japan, the test has received manufacturing and marketing approval from the Ministry of Health, Labour and Welfare as the Oncotype DX Breast Recurrence Score Program and is covered by National Health Insurance.

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